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SECTOR AT A GLANCE 02

Health and Family Welfare in India

Health Sector in India

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The '**Sector-at-a-Glance**' provides an easy-to-read overview and basic understanding of a sector, sub-sector, or thematic area. The facts and insights on the sector are supported by relevant data and best practices from credible government and multilateral sources. The document covers the structure, key trends, opportunities and challenges, government initiatives, and the way forward for the sector.

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Overview

The government of India (GoI) is committed to providing the highest possible level of health and well-being for all, at all ages, through a preventive and promotive healthcare orientation in all developmental policies and universal access to high-quality healthcare services without any financial burden. This is being accomplished by expanding access, raising the standard of care, and lowering costs associated with healthcare delivery.¹

The government of India has undertaken several initiatives to promote Universal Health Coverage (UHC) and meet the standards of Sustainable Development Goal 3 (SDG 3) of "Good Health and Well-being", one of the 17 Sustainable Development Goals established by the United Nations in 2015.^{1,2}

The **Ministry of Health and Family Welfare** (MoHFW), a Ministry under the Government of India is charged with the formulation, implementation, and execution of a variety of national health and disease control programmes, missions, schemes, and new initiatives under the realm of health and family welfare.



MoHFW: Departments and Offices³

Ministry of Health and Family welfare has two Departments viz.

- **Department of Health and Family welfare**
- **Department of Health Research**

Ministry of Health and Family welfare has following two attached offices;

- **Directorate General of Health Services (DGHS)** is an attached office of the Department of Health and Family welfare and has subordinate offices all over the country. It renders technical advice on all Medical and Public Health matters and is involved in the implementation of various Health Services.
- **National Health Authority (NHA)** is an attached Office to Ministry of Health and Family welfare. NHA serves as the main body responsible for implementing "Ayushman Bharat- Pradhan Mantri Jan Arogya Yojana" and responsible for developing strategy, building technological infrastructure, and implementation of "National Digital Health Mission" for creating a National Digital Health Eco-system.



MoHFW: Vision¹

Attainment of highest possible level of health and well-being for all, through preventive and promotive health care and universal access to good quality health services without anyone having to face financial hardship as a consequence.



MoHFW: Mission¹

- To ensure availability of quality healthcare on equitable, accessible and affordable basis across regions and communities with special focus on under-served population and marginalized groups.
- To establish comprehensive primary healthcare delivery system and well-functioning linkages with secondary and tertiary care health delivery system.
- To develop the training capacity for providing human resources for health (medical, paramedical and managerial) with adequate skill mix at all levels.
- To regulate health service delivery and promote rational use of pharmaceuticals in the country.



MoHFW: Objectives¹

- To improve health status of the people through concerted action.
- To expand preventive, promotive, curative, palliative and rehabilitative services provided through public health sector with focus on quality.
- To progressively achieve Universal Health Coverage.

¹ Ministry of Health and Family Welfare.(2019). *Vision, Mission and Objective Statement for MoHFW*.

² Ministry of Health and Family Welfare. (2017). *National Health Policy 2017*.

³ Ministry of Health and Family Welfare. (n.d). *About the Ministry*. Retrieved 1 September 2023.

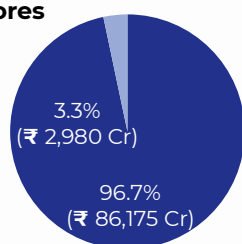
⁴ Ministry of Health and Family Welfare. (2023). *Detailed Demand for Grants 2023-24*.

Ministry of Health and Family Welfare Budget

Ministry's Budget allocation has been increasing gradually and focusing on strengthening existing health systems and establishing new health facilities.

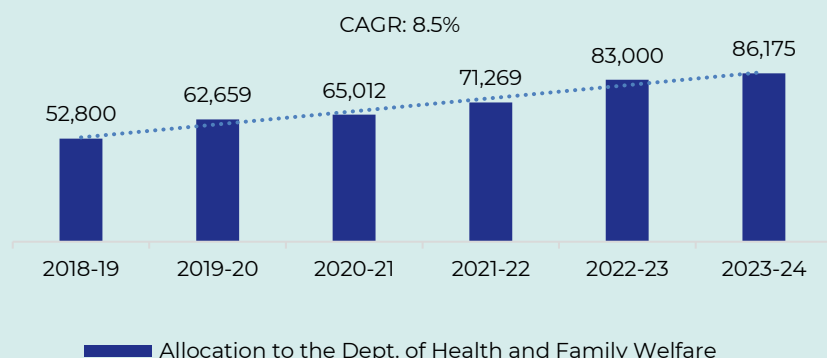
Fig 1 - MoHFW Budget 2023-24⁴

For 2023-24, 96.7% of budget was allotted to Department of Health and Family from the total budget of **INR 89,155 Crores**



- Department of Health & Family Welfare
- Department of Health Research

Fig 2 - Budget allocation to the Department of Health and Family Welfare over Years (in INR Cr)^{4,5}



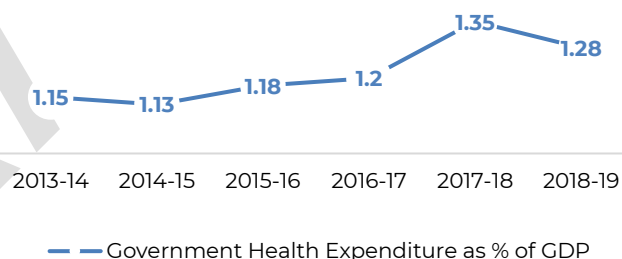
Health Financing



Government Health Expenditure (GHE)⁶

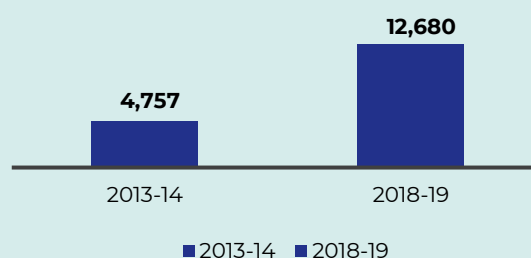
- GHE has shown a positive trend rising from 1.15% in 2013-14 to 1.35% in 2017-18 and then declining to 1.28% in 2018-19.
- Government Health Expenditure includes all schemes funded and managed by Union, State, and Local Governments including quasi-Governmental organisations and donors in case funds are channelled through Government organisations.

Fig 3 - Government Health Expenditure as Percentage of GDP 2013-14 to 2018-19⁶



- External funding has decreased significantly and government financed health insurance expenditure increased by 167% from 2013-14 to 2018-19.⁶
- Government Health Insurance Schemes include:⁶
 - ✓ Employees' State Insurance Scheme (ESIC)
 - ✓ Central Government Health Scheme (CGHS)
 - ✓ Ex-servicemen Contributory Health Scheme (ECHS)
 - ✓ Government-based voluntary insurance schemes like PMJAY, RSBY, state specific government health insurance schemes etc.

Fig 4 - Government Financed Health Insurance Expenditure in Cr⁶



⁵ Ministry of Health and Family Welfare. (2022). *Detailed Demand for Grants 2021-22*.

⁶ Ministry of Health and Family Welfare. (2023). *National Health Accounts Estimates for India 2019-20*.

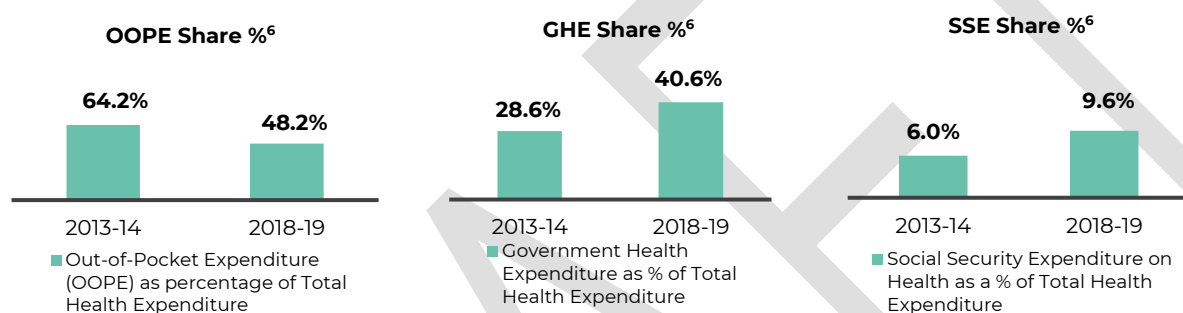
Health Expenditure

For the year 2019-20, Total Health Expenditure (THE) for India is estimated to be INR 6,55,822 Crores which includes current and capital expenditures incurred by Government and Private Sources including External/Donor funds⁶

Fig 5 - Total Health Expenditure Share in Percentage (2013-2019)⁶

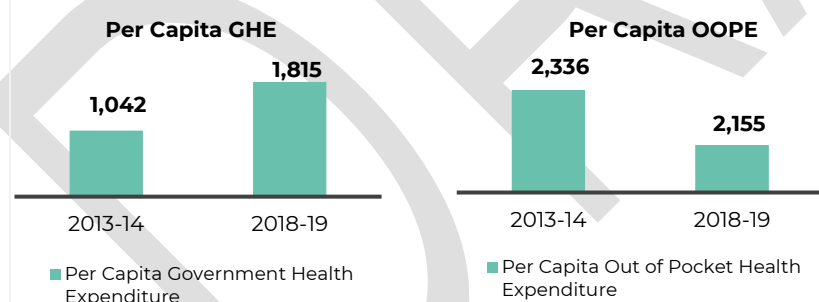
(Out of Pocket Expenditure, Government Health Expenditure, Social Security Expenditure)

- Out of Pocket Expenditures (expenditures directly made by households at the point of receiving health care) has significantly reduced from 64.2% in 2013-14 to 48.2% in 2018-19.
- Similarly, Government's contribution in total health expenditure has increased to 40.6% in 2018-19.
- Social Security Expenditure and Government Health Expenditure as a percentage of Total Health Expenditure has increased simultaneously reflecting government's growth in health expenditure.



Per Capita Health Expenditure⁶

Fig 6 - Per Capita OOPE and GHE



- Government's per capita spending on health has risen 74% since 2013-14, i.e., INR 1,042 to INR 1,815 in 2018-19.
- There was an 8% decrease in Per capita out-of-pocket expenditure from INR 2,366 in 2013-14 to INR 2,155 in 2018-19.

Current Health Expenditure⁶

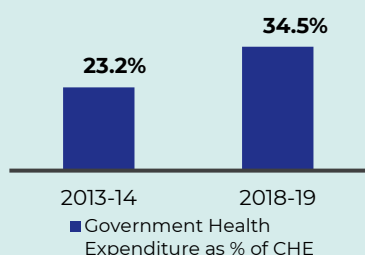
Current Health Expenditure defined as the final consumption expenditure of healthcare goods/services net capital expenditures. Current Health Expenditure (CHE) is INR 5,93,659 crores (90.52% of THE) and capital expenditures is INR 62,163 crores (9.48% of THE).

Fig 7 - Current Health Expenditure Share in Percentage (2013-2019)

CHE depicted similar trend of decrease in the out-of-pocket expenditure and increase in government health expenditure

Primary Health care is attributed of having 55.2% share of the current health expenditure in 2018-19 rising from 51.1% in 2013-14

GHE as Percentage of CHE⁶



OOPE as Percentage of CHE⁶

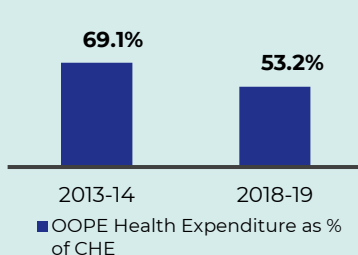


Fig 8 - Share of Primary Health Care in Current Government Health Expenditure in Percentage

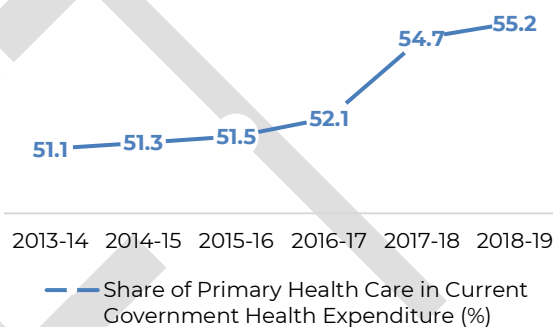
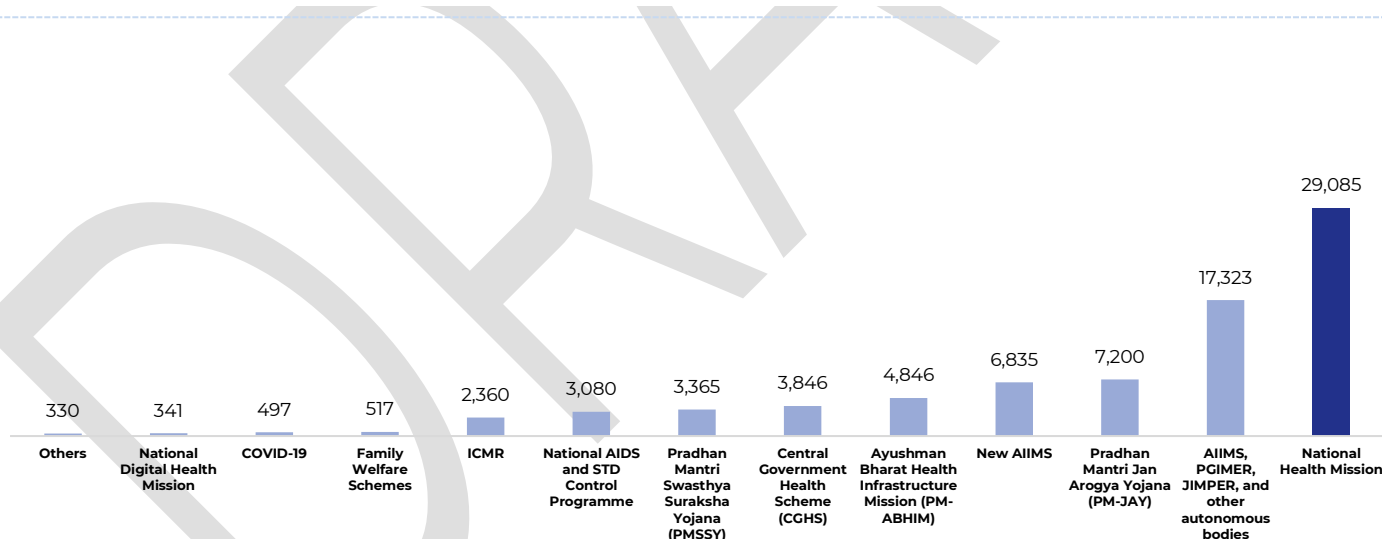


Fig 9 - Main heads of expenditure for Ministry of Health and Family Welfare (in crores, 2023-2024)⁵



In 2023-24, major budget (33%) has been allocated towards the National Health Mission

National Health Mission (NHM)⁷

National Health Mission (NHM) encompasses its two Sub-Missions, National Rural Health Mission (NRHM) and National Urban Health Mission (NUHM). Government of India launched the National Health Mission with an aim of universal access to equitable, affordable, and high-quality health-care services that are accountable and responsive to people's needs. In 2013, National Urban Health Mission was launched as a Sub-mission, with National Rural Health Mission launched in 2005 being the other Sub-mission of National Health Mission. Health System Strengthening, Reproductive-Maternal-Neonatal-Child and Adolescent Health (RMNCH+A), and Communicable and Non-Communicable Diseases are the key programmatic components.

Implementation Strategy:

NHM provides financial and technical assistance to all levels of health care institutions, from primary care to district hospitals, in order to deliver accessible, affordable, accountable, and effective quality healthcare, particularly to the poor and vulnerable segments of the population.

The mission seeks to bridge the rural healthcare service gap through improved health infrastructure, human resource augmentation, and improved service delivery in rural areas, as well as programme decentralisation to the district level to facilitate need-based interventions, improve intra and inter-sectoral convergence, and effective resource utilisation.



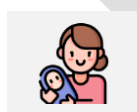
INR 29,085 Crores

Budget allocated in FY 2023-2024 for NHM (excluding AYUSH mission and Senior Citizen Health Insurance Scheme)⁵

NHM Financing Components⁷

- NRHM-RCH Flexipool
- NUHM Flexipool
- Flexible pool for Communicable disease
- Flexible pool for non-communicable disease including Injury and Trauma
- Infrastructure Maintenance
- Family Welfare Central Sector component

NHM Key Targets²



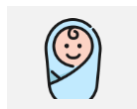
Reduce MMR to 90 per lakh live births by 2025



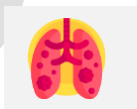
Reduce OOPE



Annual Malaria Incidence to be <1/1000 by 2025



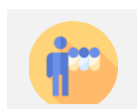
Reduce IMR to 23/1000 live births by 2025



Ending the TB epidemic by 2025



Prevention and reduction of anemia in women



Sustain TFR to 2.1 by 2025



Reduce prevalence of Leprosy to <1/10000 population and incidence to zero in all



Reduce mortality and morbidity from NCDs and CDs

NHRM-RCH: National Rural Health Mission-Reproductive Child Health; MMR: Maternal Mortality Ratio; IMR: Infant Mortality Rate; TFR: Total Fertility Rate; OOPE: Out of Pocket Expenditure; TB: Tuberculosis; NCDs: Non-Communicable Diseases; CDs: Communicable Diseases

⁷ Ministry of Health and Family Welfare. (n.d). *National Health Mission*.

Major Achievements and Progress Under National Health Mission



India has reached a replacement level of Total Fertility Rate (TFR), which is now at 2.0⁹



India has met its MMR target of less than 100 per lakh live births and is on track to meet the SDG target of less than 70 per lakh live births by 2030⁸



India's infant mortality rate (IMR) has decreased to 28 per 1,000 live births in 2020 from 30 per 1,000 live births in 2019 and aim to further reduce it to 23 per 1,000 Live birth¹⁰



India has recorded a 25.24% decrease in malaria cases and a 4.67% decrease in malaria deaths in 2020 compared to 2019¹¹



Leprosy prevalence has decreased and 610 districts had achieved prevalence rate of less than 1 per 10,000 population¹²



In 2021, the TB treatment success rate was 84%, and in 2022 (January-September), it further increased to 84.2%¹³



Percentage of Kala Azar (KA) endemic blocks with one KA case per 10,000 grew from 74.2% in 2014 to 97.5% in 2020-21¹⁴

8 Census India. (2020). *Special Bulletin on Maternal Mortality in India 2018-20*.

9 Ministry of Health and Family Welfare. (2021). *National Family Health Survey (NFHS-5) Compendium of Fact Sheets Key Indicators India and 14 States/UTs (Phase III)*.

10 Census India. (2020). *Sample Registration System (SRS) Statistical Report 2020*.

11 National Centre for Vector Borne Diseases Control. (2022). *Malaria Situation*.

12 Central Leprosy Division. (2023). *National Strategic Plan and Roadmap for Leprosy 2023-2027*.

13 Central TB Division. (2023). *Leading The Way. India TB Report 2023*.

14 National Center for Vector Borne Diseases Control. (n.d). *Kala Azar Situation in India*. Retrieved 1 September 2023.

Trends in Health and Family Welfare

Sub-sectoral trends are represented as various indicators encompassing health, demographic trends, education, socio economic trends.

1. Key Health Indicators

Figures related to mortality and nutrition have shown positive trends and development.



Mortality Figures

Fig 10a - MMR
(Per 1,00,000 Live Births)⁸

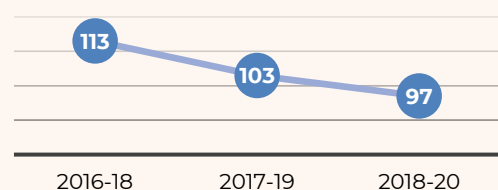
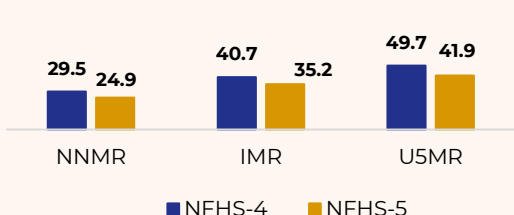


Fig 10b - NNMR, IMR and U5MR
(Per 1000 Live Births)⁹



SDG Targets by 2030

MMR
Less than
70 Per
1,00,000⁸

NMR
Less than 12
Per 1,000¹⁵

U5MR
Less than
25 Per
1,000¹⁵

- Mortality indicator related to women and child have significantly improved along with significant improvement in number of institutional births.
- Govt has implemented programmes that aligns with the SDG targets for lowering Maternal mortality ratio (MMR), Under Five Mortality Rate (U5MR), and Neonatal Mortality Rate (NMR).



Nutritional Status

Fig 10c - Anaemia Prevalence in Percentage⁹

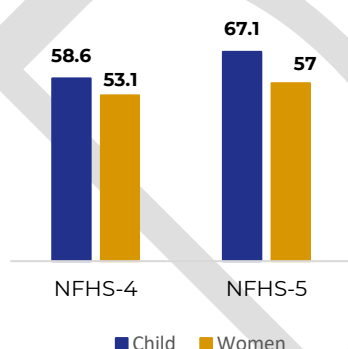


Fig 10d - NFHS-5
Overweight and Low BMI
in Percentage⁹

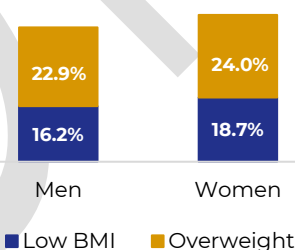
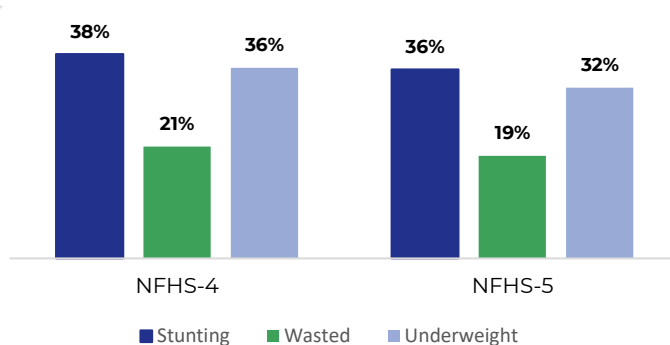


Fig 10e - Nutritional Status of Children⁹



- Anaemia prevalence among women aged (15-49 years) grew from 53.1% in 2015-2016 to 57% in 2019-2021.⁹
- 67.1% of children (6 months - 59 months) were found to be anaemic in the NFHS-5 survey.⁹
- 22.9% and 24% population of men and women were found to be overweight in NFHS-5 survey, which is gradually becoming a rising concern along with malnutrition.⁹
- Indian children suffer from triple burden of stunting, wasting, and underweight. 19% of children within the age group of 0 to 5 years are still suffering from wasting as per NFHS-5 survey.⁹

¹⁵ Ministry of Health and Family Welfare. (n.d). *National Health Mission Child Health*. Retrieved 1 September 2023.

2. Demographic Indicators

Fig 11a - Population Trend in Crore¹⁶

*E: Estimated

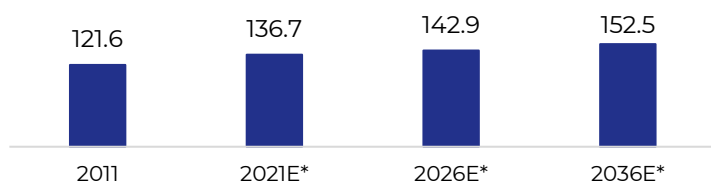
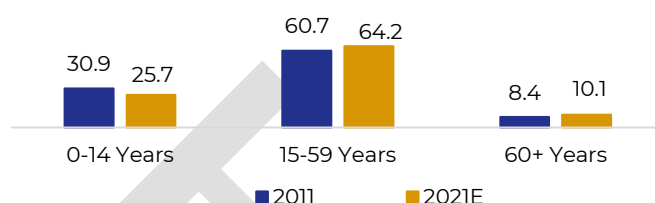


Fig 11b - Percentage Distribution of Estimated Total Population by Age-Group Years (2011 and 2021)¹⁶



- Demographic dividend or working age population (15-59) has grown from 60.7% in 2011 to 64.2% in 2021.¹⁶
- Dependency ratio reduced from 64.6 in 2011 to 55.7 in 2022 (The dependency ratio is a measure of the number of dependents aged zero to 14 and over the age of 65, compared with the total population aged 15 to 64).¹⁶
- Life expectancy has risen from 59.4 years in 1991 to 70 years 2020 indicating enhancement in health facilities.¹⁸



Sex Ratio (2021)¹⁶
945 Female per 1,000 Males



Life Expectancy (2016-220)¹⁸
70 Years



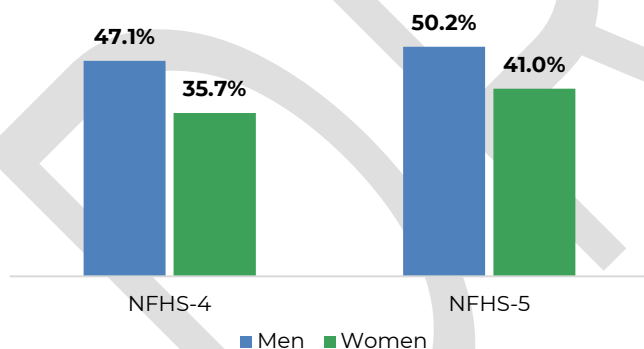
Crude Birth Rate (2020)¹⁷
19.5 per 1,000



Crude Death Rate (2020)¹⁷
6 per 1,000

3. Education and Socio-Economic Indicators

Fig 12a - Population with 10 or more Years of Education⁹



- Population with more than 10 years of education have grown for both men and women as per NFHS-5 data.⁹
- Per capita income has risen to the levels of INR 1.5 Lakhs in 2021-22 from INR 1.15 Lakhs in 2017-18.¹⁶
- Poverty incidence has decreased overall, however huge disparity still remains between rural and urban.¹⁹

Fig 12b - Annual Per Capita Net National Income in L Rupees (2017 to 2022)⁹

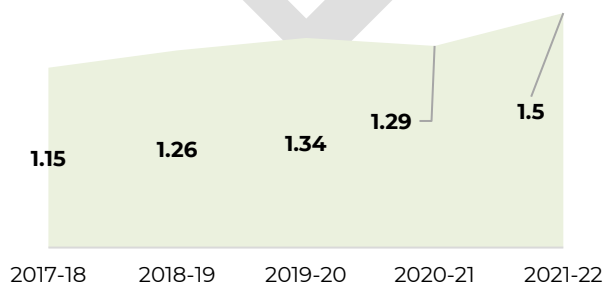
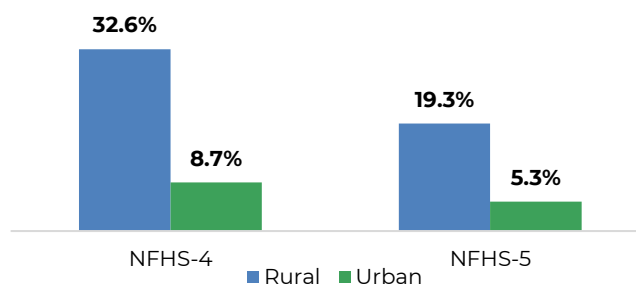


Fig 12c - Headcount Ratio (Poverty Incidence)¹⁹



¹⁶ Central Bureau of Health Intelligence. (2022). *National Health Profile 2022 17th Issue*.

¹⁷ Office of the Registrar General and Census Commissioner India. (2022). *Sample Registration System (SRS) Statistical Report 2020*. Retrieved 1 September 2023

¹⁸ Census India. (2022). *Sample Registration System Abridged Life Tables 2016-2020*.

¹⁹ United Nations Development Programme. (2023). *National Multidimensional Poverty Index - A Progress Review 2023*.

4. Health Finance Indicators

Households with any usual member covered under a health insurance/financing scheme increased from 29% to 41%.⁹

Fig 13a - Government Health Expenditure as a Percentage of GDP⁶

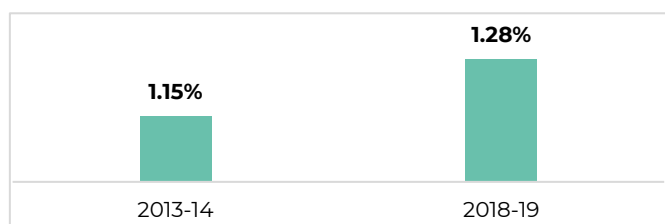


Fig 13b - Government Health Expenditure (GHE) as percentage of THE⁶



Fig 13c - Government Health Expenditure Per Capita in Rupees⁶

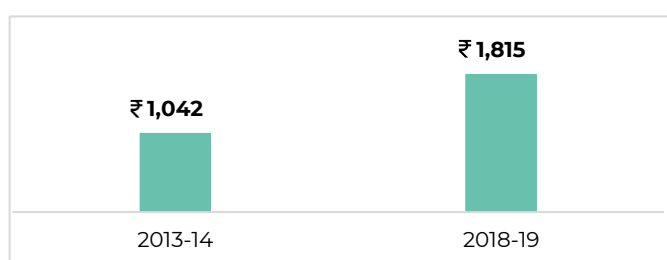
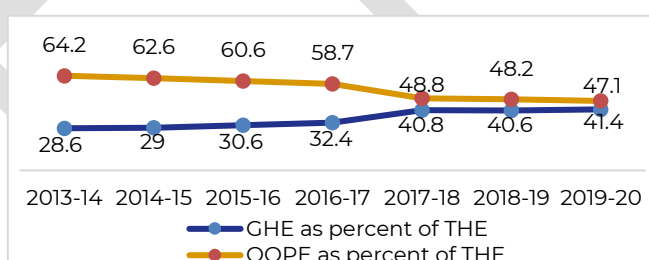


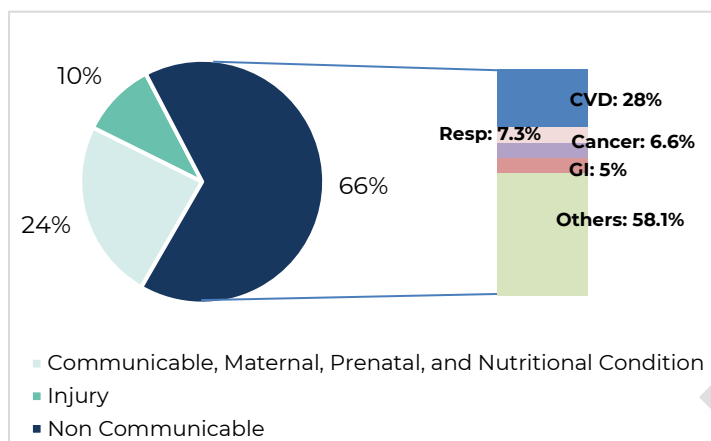
Fig 13d - Government Health Expenditure (GHE) and Out of Pocket Expenditure (OOPE) as percentage of Total Health Expenditure (THE)⁶



- Government's expenditure on health was 1.28% of its GDP in 2018-19 as compared to 1.15% in 2013-14.⁶
- Similarly, government's share of health expenditure in total health expenditure has also increased from 28.6% in 2013-14 to 41% in 2018-19.⁶
- There has been an increase of 74% in per capita expenditure in health by the government, having risen to INR 1,815 as compared to INR 1,042 in 2013-14.⁶
- There was a significant decline in out-of-pocket expenditure which reduced by 17.1% in 2019-20 in comparison to 2013-14.⁶
- National Health Accounts also reported that the government-financed health insurance expenditure has increased by 167% since 2013-14, rising from INR 4,757 Crore to INR 12,680 Crore in 2018-19.⁶

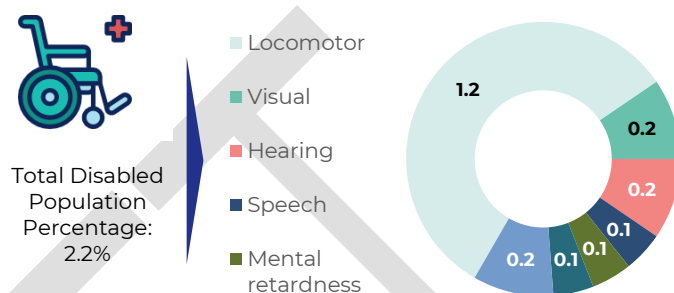
5. Health Status Indicators

Fig 14a - Causes of Death in Percentage, 2019^{16,20-22}



CVD: Cardiovascular Diseases; Resp: Respiratory diseases; GI: Gastro-Intestinal (Digestive Diseases)

Fig 14b - Percentage of persons with only one broad type of disability²³



- Mortality from NCDs has grown considerably and doubled to 66% in 2019 from 37.1% in 1990.²¹
- For vector-borne diseases, in 2021, Malaria (1.58 Lakh cases) and Dengue (1.88 Lakh cases) had the highest burden of disease while Dengue was the topmost cause of death.¹⁶
- In 2017, India had 197.3 Mn people with mental disorders, accounting for 14.3% of the country's total population.²⁴

²⁰ The World Bank. (n.d). *Cause of death, by communicable diseases and maternal, prenatal and nutrition conditions (% of total) - India*. Retrieved 3 September 2023.

²¹ The World Bank. (n.d). *Cause of Death, by Communicable Diseases (% Of Total) - India*. Retrieved 2 September 2023.

²² The World Bank. (n.d). *Cause of death, by injury (% of total) - India*. Retrieved 2 September 2023.

²³ Ministry of Statistics and Programme Implementation. (2019). *Persons with Disabilities in India NSS 76th round (July - December 2018)*.

²⁴ Dandona. (2020). *The burden of mental disorders across the states of India: The global burden of disease study 1990-2017*

6. Healthcare Delivery Indicators



Medical Doctors (2019)
0.93 per 1,000 population²⁵



Nurses and Midwives (2019)
2.39 per 1,000 population²⁵



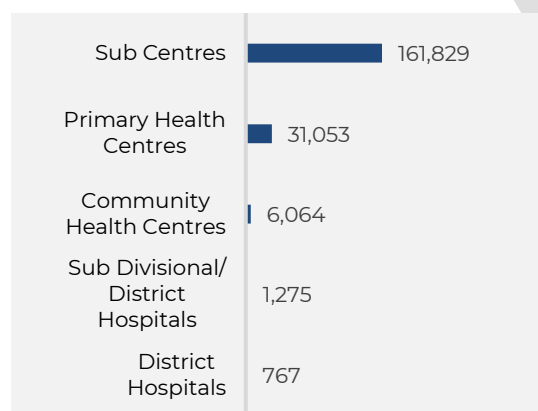
Pharmacists (2019)
0.88 per 1,000 population²⁵



Dentists (2019)
0.2 per 1,000 population²⁵

- Overall health worker density in India per 1,000 population increased from 2.9 health workers per 1,000 population in 2016 to 4.4 per 1,000 population in 2019. However, service delivery is still affected by inadequate strength of healthcare workers.²⁵
- Doctor patient ratio has increased in the last three years. However, unequal distribution of healthcare workers still exists between urban and rural areas.²⁵

Fig 15a. **Number of Health Facilities (2022)**²⁶



- Between 2005 and 2022, number of centres increased by:²⁶
 - ✓ Sub-centres (SC): 11,909
 - ✓ Primary Health centres (PHC): 1,699
 - ✓ Community health centres (CHC): 2,134
- Allopathic doctors number increased by 50.9% at PHCs from 20,308 in 2005 to 30,640 in 2022²⁶
- As of 2022 – 25,383 SCs and 3,833 PHCs are operational in the tribal areas representing India's effort to reach more and more underserved areas²⁶

Fig 15b. **Shortfall in Healthcare Manpower at SCs and PHCs in Percentage (2021-22)**²⁶

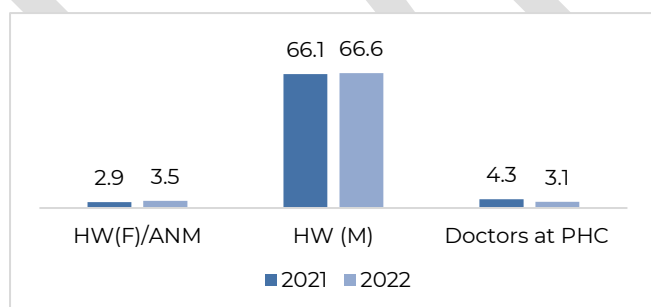
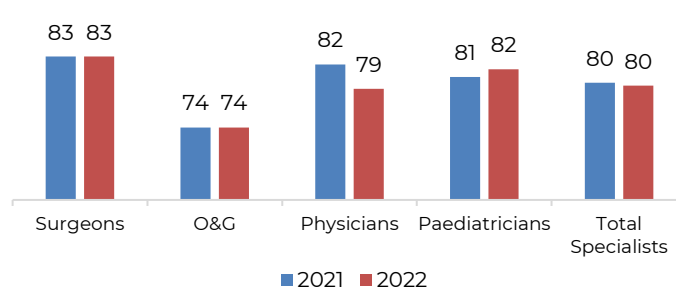


Fig 15c. **Shortfall in Healthcare Manpower at CHCs in (2021-22)**²⁶



- There is a shortfall of 3.1% in allopathic doctors at PHC, of the total requirement at all India level.²⁶
- Overall, 67.8% of the sanctioned posts of specialists at CHCs are vacant and there is a shortfall of 79.5% specialists at the CHCs as compared to the requirement.²⁶

HW(F): Health Workers Female, HW(M): Health Workers Male, ANM: Auxiliary Nurse Midwife

²⁵ World Health Organisation. (2022). *India Health System Review*.

²⁶ Ministry of Health and Family Welfare. (2022). *Rural Health Statistics 2021-22*

Opportunities and Challenges

Opportunities



Favourable investment policies²⁷

- 100% Foreign Direct Investments through automatic route are allowed for hospitals, healthcare, and medical devices.
- These investments can also support training and capacity-building programmes for local healthcare professionals, leading to more skilled workforce.



Employment generation potential

- Healthcare sector in India became the fifth largest employer in 2015*, employing 0.47 Crore people directly.²⁷
- During the 3rd round of the Quarterly Employment Survey (2021), there was a notable increase in the number of workers in the health sector. The proportion of total workers employed rose from 8.4% in April 2021 to 10.4% in October 2021.²⁸

*Data is mentioned in NITI Aayog's 2021 report



'Heal in India' attracting more value to Indian medicines²⁹

- Indian healthcare ecosystem provides treatment across modern medicine, ayurveda, yoga, and other traditional healthcare methods.
- It offers tertiary and quaternary health services, treatment for NCDs, and comprehensive rehabilitation in all major medical specialties, including cardiac care, orthopaedics, etc.



Enhanced coverage under PMJAY's

- For 2023-2024, PMJAY has been allocated ₹7,200 Cr, representing a 12% rise compared to the revised estimates of 2022-2023.⁴
- The latest Health Benefits Package (HBP 2.2) has 1,121 treatment packages within which 1,949 procedures across 27 specialties are available.³⁰
- 26,054 hospitals have been empanelled and over 21.81 Cr e-cards have been issued.³⁰



Leveraging Technology to improve accessibility³¹

- Govt has established a network of telemedicine services like Hub and Spoke e-Sanjeevani, Tele Manas (teleconsultation for mental health), Nikshay (a web-based TB registration portal), etc., which have improved health outcomes.
- Similarly, platforms like Integrated Government Online training (iGOT), Training Management Information System, Mobile Academy, etc. are being used for upskilling healthcare workers.



Increased Inter Sectoral Convergence³²

- Inter sectoral convergence was initiated under NHUM with an aim to improve the effectiveness and utilization of all national health programmes. Such practices can be scaled across different levels.
- Telangana government was able to strengthen their Mahila Arogya Samiti through collaboration with Swachh Bharat Mission and National Urban Health Mission.

²⁷ NITI Aayog. (2021). *Investment Opportunities in India's Healthcare Sector*.

²⁸ Ministry of Labour and Employment. (2022). *Labour and Employment Statistics 2022*.

²⁹ Ministry of Health and Family Welfare. (n.d). *Heal in India*. Retrieved 2 September 2023.

³⁰ Ministry of Health and Family Welfare. (2023). *Annual Report 2022-23*.

³¹ Narain, J. P. (2016). Public health challenges in India: Seizing the opportunities.

³² National Health Systems Resource Centre. (n.d). *Inter Sectoral Convergence under NUHM*.

Challenges



Budget Allocation: Bridging Gaps in Health Policy Targets:

- National Health Policy 2017 document suggested that the government's overall spending on public health should reach 2.5% of the GDP by 2025.³³
- NHA estimates for 2019-20 reported Government Health Expenditure (GHE) to be 1.35% of GDP, reflecting the ongoing need for continued efforts to align budget allocations with evolving healthcare requirements.⁶



Primary healthcare infrastructure needs more improvement:

- Number of centres has increased but not as much as projected population growth.³⁴
- There is shortfall in primary health care - SCs have a shortfall of 25%, PHCs of 31%, and CHC of 36%.³⁴
- As per the RHS 2021-2022 report, India's 6,064 Community Health Centres (CHCs) are experiencing a significant shortage of approximately 80% in the number of required surgeons and paediatricians.²⁶



India faces double burden of NCDs and CDs

- Many diseases have been eradicated, including polio, guinea worm sickness, yaws, and tetanus.³¹
- Despite these advances, communicable diseases like (HIV/AIDS), tuberculosis (TB), etc. remain a significant public health issue.³¹
- In addition, noncommunicable diseases (NCDs) are a leading cause of mortality in the nation, accounting for 66% of total deaths.²¹



Health treatment is expensive

- Out of pocket expenditure has come down significantly in recent years, yet people are still spending around 47.1% from their own pocket for treatment.⁶
- Cancer care is provided in only 50 public hospitals vs 200 private hospitals – forcing people to resort to high-cost cancer treatment in private hospitals.³⁴



Healthcare workforce is lower than global average

- Indian healthcare workforce is rapidly rising however population density of allopathic doctors is currently lower than global average
- Number of registered allopathic doctors are 13,00,290.¹⁶
- Additionally, total number of registered nurses in India are 35,14,373.¹⁶



Geographical remoteness and limited infrastructure Tribal Health³⁵

- In 50% of Indian States, there is a shortfall of health facilities (27%-40%), in tribal areas.
- Along with infrastructure shortfall, retaining health care workers in tribal areas is also challenging.
- Furthermore, lack of data regarding tribal population has limited government to effectively draft policy to uplift the community.

NHP – National Health Policy, WHO – World Health Organisation, SC – Sub-Centres, PHC – Primary Health Centres, CHC – Community Healthcare Centres, SECC - Socio-Economic Caste Census, RHS - Rural Health Statistics

³³ India Budget. (2023). *Social Infrastructure and Employment: Big Tent.*

³⁴ Rajya Sabha. (2022). *Report No. 139 of the Standing Committee on Health and Family Welfare.*

³⁵ National Health Systems Resource Centre. (2022). *Report of the Expert Committee on Tribal Health.*



Challenges arising from Demographic Transition³⁶

- **Rising Old Age:** It is estimated that more than 30% of elderly individuals live below the poverty line, while an additional 33% are vulnerable to poverty. A majority of these seniors, ~80%, reside in rural areas, and a significant portion, about 73%, lack literacy.
Increasing elderly population necessitates an expanded availability of elderly care services, encompassing options such as nursing homes, day-care centers, or specialized intensive care tailored to the specific health needs of this demographic.
- **Urban Poor Population:** Healthcare delivery system poses numerous challenges for the urban impoverished population in India. Prevalence of communicable diseases like tuberculosis and NCDs, such as diabetes, is significantly elevated among urban poor communities when contrasted with their rural counterparts. ~35% of urban households manage to sustain themselves with less than one dollar per person per day. Situation becomes even more complex in disease control efforts due to the substantial population of migrant laborers and daily wage workers in major cities.

Cross-cutting Themes

Climate



Climate change is anticipated to have severe health impacts in India, with projections indicating that by 2100, an estimated 0.15 crore additional deaths per year could occur due to climate-related factors, comparable to the current death rate in India caused by all infectious diseases combined.³⁷



Climate change poses a significant threat to food security and malnutrition in India, with 6.5 Cr people at risk of starvation and an estimated 1.7 Crore in India facing famine by 2030.³⁸



Losses to health infrastructure are on the rise, with India consistently experiencing high fatalities from natural disasters. The 2018 Kerala floods damaged a 125-year-old hospital serving 350,000 people, destroying medicines worth INR 16.84 Crore and equipment, including dialysis and ultrasound machines worth INR 28 Crore.³⁹



Respiratory infections, chronic obstructive pulmonary disease, pneumothorax, asthma, allergies, hyperthermia, and dehydration are too caused by climate change. In India, around 6% of children are susceptible to respiratory tract infections, and approximately 2% of adults suffer from asthma.⁴⁰



An analysis of the heat index (HI) in conjunction with the Climate Vulnerability Index (CVI) indicates that over 90% of the nation is facing high levels of risk, significantly affecting adaptive livelihood capacity, food grain yield, the spread of vector-borne diseases, and urban sustainability.⁴¹

³⁶ Center for Policy Impact. (2022). *India's Health Transitions*.

³⁷ Climate Impact Lab. (2019). *Climate Change and Heat-Induced Mortality in India*.

³⁸ World Wildlife Fund. (2022). *Global food policy report: Climate change and food systems*.

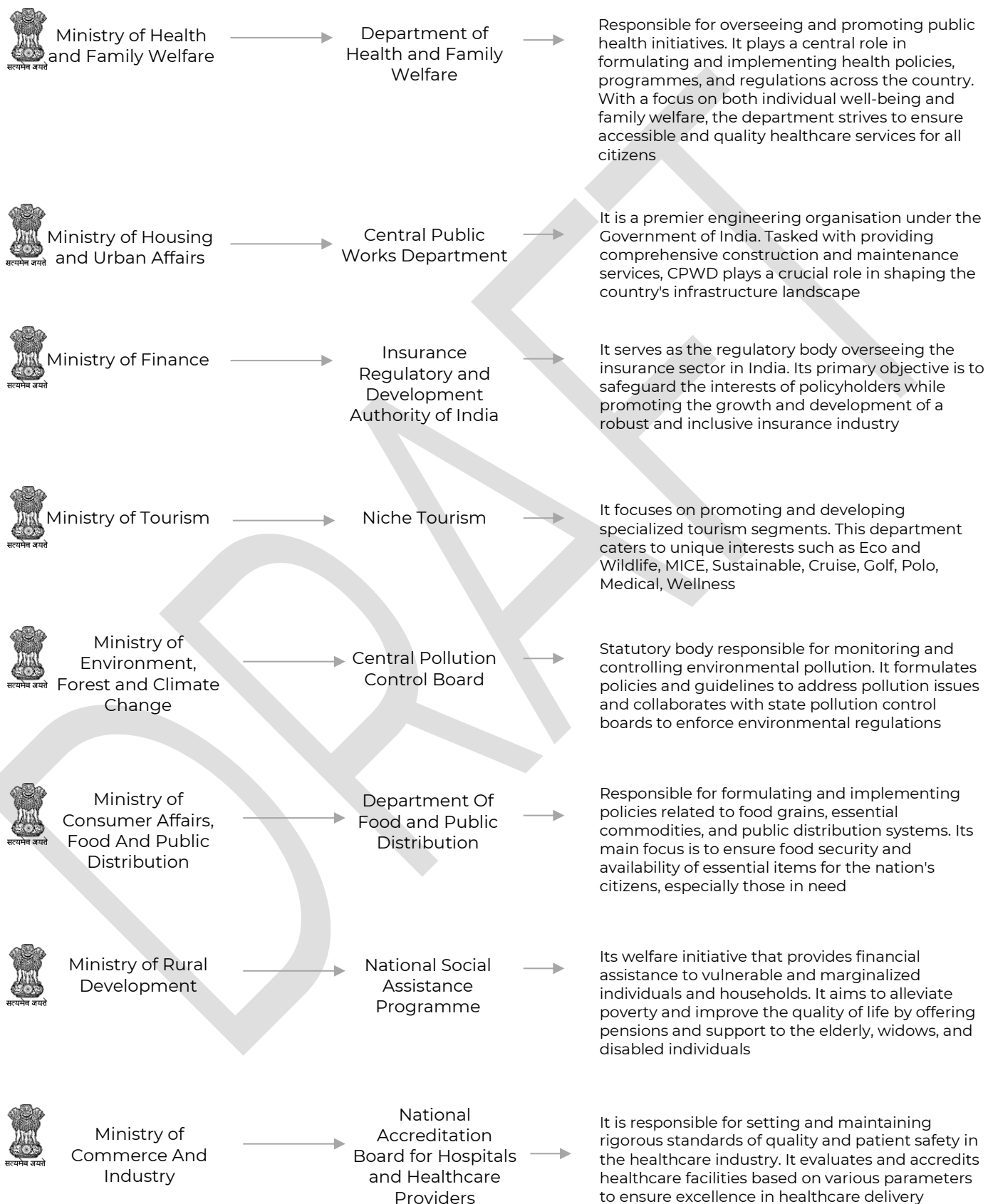
³⁹ NDTV. (2018). *125-Year-Old Hospital, Among The Best In Country, Ruined In Kerala Floods*. Retrieved 17 August 2023.

⁴⁰ Pandey P. (2021). *Air Pollution, Climate Change, and Human Health in Indian Cities: A Brief Review*, 2021.

⁴¹ Bardhan R. (2023). *Lethal heatwaves are challenging India's sustainable development*, 2023.

Ministries and Departments

Inter-ministerial and departmental collaboration is an important aspect of governance in India. Various ministries, departments, and organisations work together to achieve common goals and objectives.

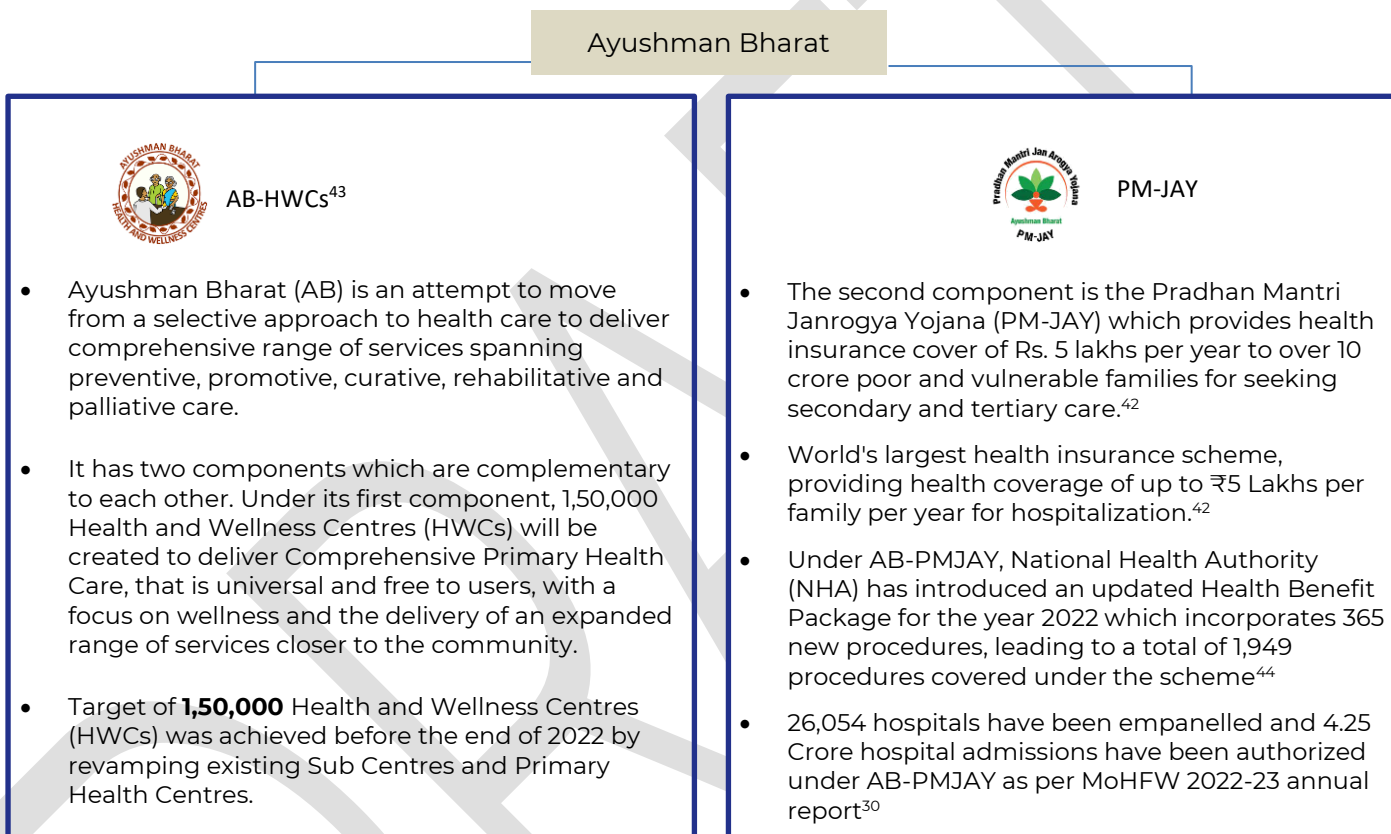


Government Interventions

Ministry of Health oversees implementation of multiple initiatives directed at improving health status of India.

Ayushman Bharat Programme

Ayushman Bharat was launched in 2018 as recommended by the National Health Policy 2017, to achieve the vision of Universal Health Coverage. The initiative is designed to meet Sustainable Development Goals and its underlining commitment, to "leave no one behind".⁴²



Functional HWCs ⁴⁵					
1,61,230*	Sub Health Centre – HWC	Primary Health Centre – HWC	Urban Primary Health Centre – HWC	Urban Health and Wellness Centre – HWC	AYUSH Centre – HWC
As on 24 August 2023	1,21,095	23,541	4,808	8,082	3,704

ABHIM: Ayushman Bharat Health Infrastructure Mission, ABDM: Ayushman Bharat Digital Mission, PMJAY: Pradhan Mantri Jan Arogya Yojana, HWCs: Health and Wellness Centres

⁴² National Health Authority. (n.d). *About PMJAY*. Retrieved 1 September 2023

⁴³ National Health Mission. (n.d). *AYUSHMAN BHARAT: Comprehensive Primary Health Care through Health and Wellness Centres*.

⁴⁴ National Health Authority. (2022). *PMJAY Annual Report 2021-22*.

⁴⁵ Ministry of Health and Family Welfare. (n.d). *Ayushman Bharat – Health and Wellness Centre*. Retrieved 19 September 2023.

Ayushman Bharat: Other Components



Ayushman Bharat Digital Mission^{46,47}

- Ayushman Bharat Digital Mission aims to strengthen the accessibility of healthcare with the help of information technology and associated technologies supporting the healthcare systems in a “citizen-centric approach”.
- Under the National Health Policy 2017, the MoHFW developed the National Health Digital Blueprint (NHDB), to lay out the plan to implement digital health holistically.
- ABDM is expected to improve the effectiveness, efficiency, and transparency of health service delivery.

It includes digital systems such as:-⁴⁶

- ✓ Ayushman Bharat Health Account (ABHA) number
 - ✓ Health Professional Registry
 - ✓ Health Facility Registry
 - ✓ ABHA Mobile App (PHR)
- As of 12th September 2023, 45,21,91,218 ABHA numbers have been created and 30,62,04,997 ABHA Linked Health Records and 2,18,972 Health facilities have been registered under ABDM.



PM-ABHIM⁴⁸

- Aims to make all districts self-reliant under this scheme in terms of infrastructure facilities and health care delivery and to build IT enabled surveillance system for disease.
- Objective of the scheme is strengthening healthcare infrastructure, surveillance, and health research across the country.

Centrally Sponsored Components

- ✓ Construction of 17,788 building less Sub Health Centres level AB-HWCs in rural areas in 7 High Focus States
- ✓ 11,024 Urban HWCs cumulatively over a five-year duration
- ✓ 3,382 BPHUs in all the Blocks of the 11 EAG States/UTs
- ✓ Integrated Public Health Laboratories in all 730 districts
- ✓ Support is available to establish 602 Critical Care Hospital Blocks



Central Sector Components

- ✓ 12 Central Institutions as training and mentoring sites
- ✓ Strengthening of the National Centre for Disease Control, 5 New Regional NC, DCs and 20 metropolitan health surveillance units
- ✓ Expansion of the Integrated Health Information Portal to all States/UTs
- ✓ Operationalization of 17 new Public Health Units and strengthening of 33 existing Public Health Units at Points of Entry
- ✓ Setting up of 15 Health Emergency Operation Centres and 2 container based mobile hospitals

⁴⁶ National Health Authority. (n.d). *Ayushman Bharat Digital Health Mission*. Retrieved 2 September 2023.

⁴⁷ National Health Authority. (n.d). *Ayushman Bharat Digital Health Mission Dashboard*. Retrieved 1 September 2023.

⁴⁸ Ministry of Health and Family Welfare. (2022). *Note on PM- Ayushman Bharat Health Infrastructure Mission*.

A. Universal Immunisation Programme

UIP targets around 3.04 Cr pregnant women and 2.67 Cr newborn each year through more than 1.2 Cr immunization sessions.³⁰

A total of 12 vaccine are provided free of cost nationally and sub-sub nationally³⁰



National

Vaccines for Diphtheria, Pertussis, Tetanus, Polio, Measles, Rubella, severe form of Childhood Tuberculosis, Hepatitis B and Meningitis and Pneumonia caused by Hemophilus Influenza type B, Rotavirus diarrhoea, and Pneumococcal Pneumonia are provided across country³⁰



Sub-National

Vaccine for Japanese Encephalitis are provided in sub nationally in endemic districts³⁰



Intensified Mission Indradhanush drive and Routine Immunization System Strengthening are two major interventions within the mission



Mission Indradhanush⁴⁹

Launched in 2014, the primary objective of this flagship programme is to enhance Routine Immunization coverage. Its focus is on regions with inadequate immunization rates, aiming to provide vaccinations to all children and pregnant women who have been left out or have dropped out.⁴⁹

Current Focus: Intensified Mission Indradhanush 4.0

- Areas with disrupted routine immunisation services due to COVID-19 pandemic including regions where immunisation sessions were not conducted due to restrictions like lockdown, high burden areas of covid etc.
- Non-migratory/ settled and migrant high-risk areas as defined in Polio Eradication activities. Areas with Tenants, families who had temporarily migrated for work, nomadic sites, Brick Kilns Construction Sites etc.
- Areas with high incidence of Measles, regions like orphanage, prisons, red-light areas, tribal areas, regions effected by natural calamities, slum areas in Urban regions, and other hard to reach and areas with vaccine hesitancy. Villages/areas with vacant sub centres are targeted and newborn who are born in home are also focused.

⁴⁹ Intensified Mission Indradhanush. (n.d). *About the Programme*. Retrieved 2 September 2023.

Progress and Way Forward⁴⁹

Since its launch in 2014, 701 districts across the country have been covered through the completion of eleven phases of the mission until February 2023. A post IMI Coverage Evaluation Survey (CES) conducted in 2018 showed an increase in immunization coverage by 18.5 percentage points.

378 districts were prioritised to implement IMI- 4.0 in 2022 and 75 other districts under the Azadi Ka Amrit Mahotsav. Out of these 75 districts, 37 districts are already included in the 378 identified IMI districts, and 38 districts are exclusive for Azadi ka Amrit Mahotsav. Thus, the total number of districts for IMI activity is 416.



Children Vaccinated*(IMI4.0): ⁵⁰
5,99,158

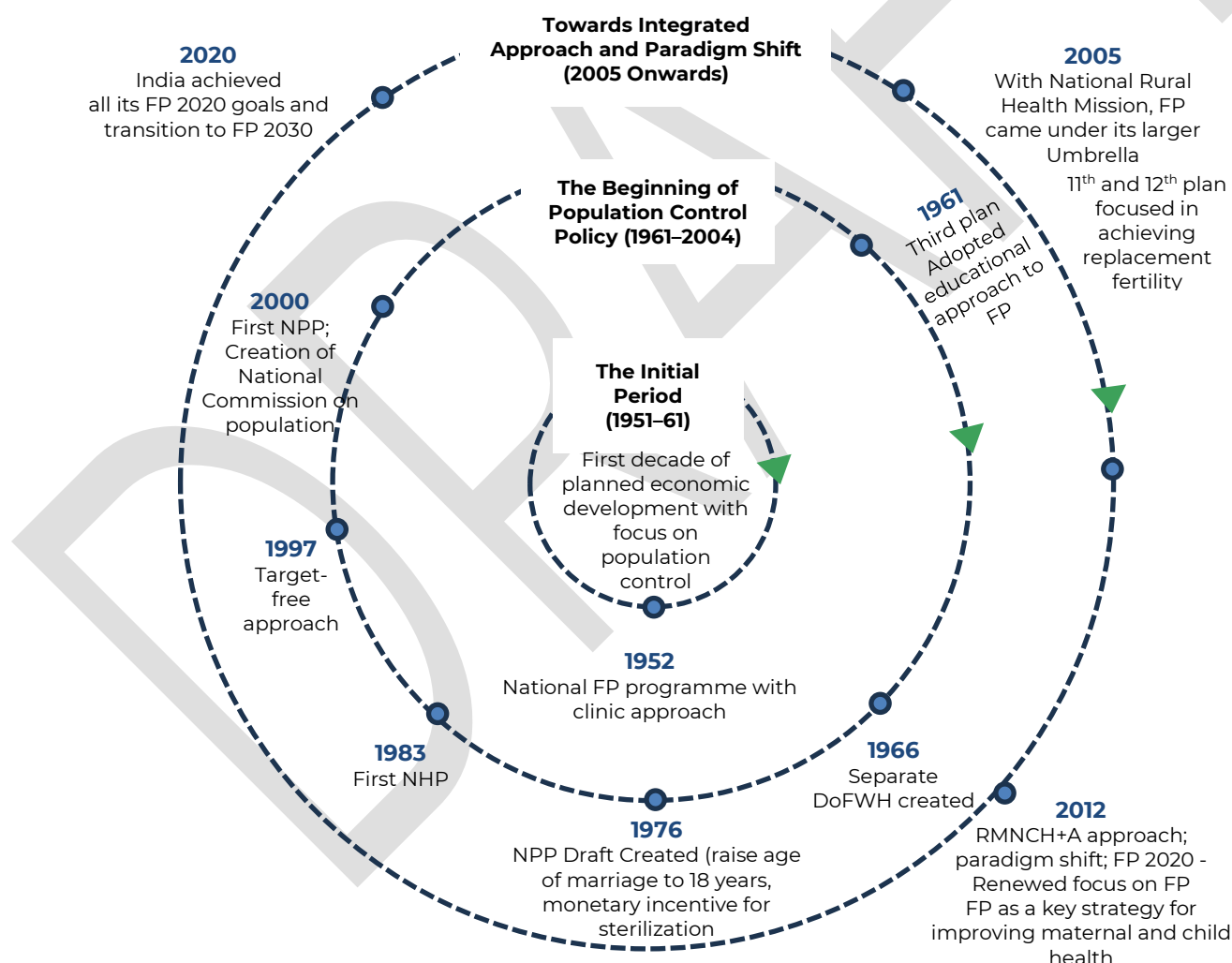


Pregnant Women Vaccinated* (IMI4.0): ⁵⁰
15,31,468

*Data as of 11th August 2023

B. Family Planning Programme⁵¹

Fig 16 - Evolution of Family Planning Programme (FP) over Years



⁵⁰ Intensified Mission Indradhanush 4. (n.d). *Intensified Mission Indradhanush 4.0*. Retrieved 2 September 2023.

⁵¹ Family Planning Division Ministry of Health and Family Welfare. (2022). *India Vision FP 2030*.

Existing Programmes⁵¹



Mission Parivar Vikas was launched by GoI in 2016 to increase access to contraceptives and family planning services in all levels of health facilities. 146 high priority districts were chosen in the 7 high focus states with TFR of 3 and above for the scheme. Initiative also aims to improve awareness through Naya Pahal Kit and Saas Bahu Sammelan.

In order to maximize supply side management for the accessibility of family planning supplies up to all levels of the health system, the **Family Planning Logistic Management Information System (FPLMIS)** was also introduced.

FP 2030 – India's Commitments⁵¹

- ▶ Exploring of introduction of new contraceptives, to increase the variety and accessibility of contraceptive alternatives by 2030.
- ▶ Ensuring healthy timing and spacing of pregnancies by improving the demand, uptake, and quality for post pregnancy contraception services.
- ▶ Scaling up of Mission Parivar Vikas (MPV) to deliver services of guaranteed quality in the most difficult-to-reach rural and urban areas by offering a comprehensive range of services at all MPV District levels.
- ▶ Intensifying Social and Behaviour Change activities (SBC) for Family Planning and strengthening access to information and services for all girls and women and couples with specific focus on young people.
- ▶ Civil society commitments for creating awareness on family planning commodities and services and mobilizing community for increasing uptake as well as providing services through civil society organisation.

C. RMNCH+A

Provisions of interventions for women, adolescent, and child health is made under RMNCH+A.⁵²

Women Health⁵³



Janani Suraksha Yojana incentivizes institutional deliveries among pregnant women, especially those from marginalized communities, to reduce maternal and neonatal mortality.



Janani-Shishu Suraksha Karyakram is aimed at providing free maternal and neonatal healthcare services to pregnant women and newborns, reducing maternal and infant mortality rates.



Pradhan Mantri Surakshit Matritva Abhiyan offers pregnant women in their second and third trimesters fixed-day, guaranteed, comprehensive, and high-quality antenatal care on the ninth day of each month.



Surakshit Matritva Aashwasan (SUMAN) provides quality healthcare services during pregnancy and childbirth, promoting maternal and child well-being at no cost and zero tolerance for denial.



LaQshya (Labour Room Quality Improvement Initiative) focuses on improving the quality of care during labour and in the immediate post-partum period. More than 2,800 health care facilities were identified for gap assessment, nearly 500 units (LR/OT) received LaQshya national certification.

⁵² National Health Mission. (n.d). *Reproductive, Maternal, Newborn, Child and Adolescent Health*. Retrieved 2 September 2023.

⁵³ National Health Mission. (n.d). *Maternal Health*. Retrieved 1 September 2023.

Adolescent Health⁵⁴

GoI in 2014 launched the nation's first comprehensive adolescent health programme named **Rashtriya Kishor Swasthya Karyakram**.

Adolescent Friendly Health Clinics (AFHCs) are the first point of contact for adolescents with primary healthcare services and provides counselling and clinical services to adolescents.

Peer Educator Programme: Four peer educators (2 male and 2 female peer educators) are selected per village or 1,000 population who conduct weekly 1–2-hour interactive sessions on adolescent health.



Rashtriya Kishor Swasthya Karyakram

Weekly Iron Folic Supplementation programme is implemented in convergence with Ministry of Education and total of 5.09 crores adolescent were covered during April 2022 to November 2022.

Scheme for Promotion of Menstrual Hygiene aims to increase awareness of menstrual hygiene among adolescent girls and the scheme provided 41.21 Lakh Sanitary Napkins during April 2022 to November 2022.

⁵⁴ National Health Mission. (n.d). *Adolescent Health*. Retrieved 1 September 2023.

Child Health⁵⁵

Child Health interventions aim to reduce child health mortality indicators by improving the health services.

Facility Based Newborn and Child Care : NHM is setting up Special New Born Care Units (SNCUs), New Born Stabilization Units (NBSUs) and New Born Baby Corners (NBCCs) at different levels for taking care of sick newborns.

Mother Newborn Care Units were setup for decongestion of SNCU for newborns who do not require intensive care and just need observational care .

Facility Based Integrated Management of Neonatal and Childhood Illness focuses on upgrading skills of health workers for managing Neonatal and Childhood mortality determinants (asphyxia, sepsis, low birth weight and pneumonia, diarrhoea, malnutrition, etc.)

Integrated Management of Neonatal and Childhood Illnesses focuses on improving health systems through Pre-service and In-service training of providers.

Home Based New Born Care is being provided by ASHA who make visits to all newborns up to 42 days of life for recording data related to weight of the newborn, ensuring BCG , 1st dose of OPV and DPT vaccination are being provided and registration of birth is done.

Home based care for young child: Additional visits are being provided by ASHA on 3rd, 6th, 9th, 12th, and 15th months with an aim to promote breast feeding, exclusive breast, adequate complementary feeding, prevention of Pneumonia and Diarrhoea.

Navjaat Shishu Suraksha Karyakram: aimed at training healthcare providers in newborn care and resuscitation with the focus on tackling of issues related to Hypothermia, prevention of Infection, breast feeding initiation, etc.

Nutrition Rehabilitation Centres have been established at public health facility level to provide medical and nutritional care to children under 5 who are severely undernourished (SAM) and have medical issues.

MusQan focuses on improving child friendly services in public health facilities through following strategies:-

- Strengthen clinical protocols and management process
- Children and parent attendant friendly ambience and infrastructure,
- Strengthen referral and follow-up services
- Provision of respectful and dignified care

Social Awareness and Actions to Neutralize Pneumonia Successfully (SAANS) seeks to raise awareness against Childhood pneumonia to enhance care seeking behaviours among parents and caregivers.

Intensified Diarrhoea Control Fortnight (IDCF) Campaign raises awareness about use of ORS and Zinc in diarrhoea. 10.2 crore children up to five years of age were provided with ORS and Zinc in 2022.⁵¹

⁵⁵ National Health Mission.(n.d). *Child Health Programmes*. Retrieved 1 September 2023.

D. Rashtriya Arogya Nidhi Scheme

Within the Rashtriya Arogya Nidhi Umbrella Scheme, individuals from economically disadvantaged families who fall below the poverty line and are afflicted with severe, life-threatening diseases are eligible to receive one-time financial support of up to ₹15 Lakh for their medical treatment at government hospitals.⁵⁶

In 2022-23, Budget allocated for RAN was ₹40 Cr, out of which, ₹2.4 Cr was released and for HMCPF (Health Minister's Cancer Patients Fund) the budget allocated was ₹40 Cr out of which ₹0.8 Cr was released.⁵⁷

Within the schemes, three components exist;



- ▶ **Rashtriya Arogya Nidhi (RAN)** – to provide financial assistance to patients suffering from life threatening diseases other than Cancer.⁵⁶
- ▶ **Health Minister's Cancer Patients Fund (HMCPF)** - to provide financial assistance to patients suffering from Cancer.⁵⁶
- ▶ Scheme for financial assistance up to ₹20 Lakh for patients suffering from specified **rare diseases**.⁵⁶

E. Poshan Abhiyan⁵⁸



Community Engagement

- **Jan Andolans** mobilises various communities and sectors to foster a desire to eat nutrient-rich food. Poshan Maah and Poshan Pakhwada, country's two largest nutrition-focused events are held through this initiative.
- **Swastha Balak/Balika Spardha** aims to identify healthy children and raise awareness of the issue and run campaigns to measure young children's growth.
- 4.35 lakh **Poshan Vatikas** have been created at AWCs to promote diet diversification and the use of healthy local produce.



Technological Intervention

- **Poshan Tracker** management application offers a 360-degree view of the Anganwadi Centre (AWC) activities, service delivery by Anganwadi Workers (AWWs), and full beneficiary management for expectant mothers, lactating mothers, and children so that results related to women's nutrition can be continuously monitored.
- Poshan Tracker has facilitated migration services for expectant and nursing mothers between AWCs inside and outside of a State.
- **To ensure last-mile tracking and service delivery**, beneficiaries are being seeded with Aadhar cards. More than 88% of recipients listed on the Poshan Tracker as of December 31st 2022.



Convergence

- **IFA supplementation, including the supply of IFA tablets, Health check-up** and referral services, and Nutrition and Health Education, are provided by establishing convergence with Ministry of Health and Family Welfare.
- Immunization in Palna homes through convergence with Mission Poshan 2.0
- Government intends to construct 50,000 AWCs building at the rate of 10,000 AWCs per year in convergence with MGNREGS under Mission Poshan 2.0.
- Increasing awareness of traditional foods for healthy pregnancy and childbirth (especially in tribal areas) in coordination with **the Ministry of Tribal Affairs**.

⁵⁶ Ministry of Health and Family Welfare. (2019). *Rashtriya Arogya Nidhi Guideline 2019*.

⁵⁷ Ministry of Health and Family Welfare. (n.d). *Demand No. 46 Department of Health and Family Welfare*.

⁵⁸ Ministry of Women and Child Development. (2023). *Annual Report 2022-23*

F. Pradhan Mantri Swasthya Suraksha Yojna

It was launched in 2006, aiming to establish advanced healthcare infrastructure in underserved regions, focusing on medical education, research, and clinical care. It strives to address regional disparities in the accessibility of affordable and dependable tertiary healthcare services, while also enhancing the resources for high-quality medical education.⁵⁹

- The scheme comprises two main aspects: Establishment of All India Institute of Medical Sciences (AIIMS) and Enhancement of existing Government Medical Colleges/Institutions (GMCIs)⁵⁹
- For 2023-24, the estimated budget for scheme is ₹3,365 Cr⁵⁷

Within the schemes, two components exist;



6 AIIMS in Phase-I: Bhopal, Bhubaneswar, Jodhpur, Patna, Raipur, and Rishikesh have fully operational AIIMs and more hospitals are being set up under different phases⁵⁹



Upgrading Existing State Government Medical colleges and institutions: A total of 75 projects/State government colleges have been approved, out of which 60 projects are already completed⁵⁹

⁵⁹ Ministry of Health and Family Welfare. (n.d) *About Scheme*. Retrieved 1 September 2023.

Way Forward



India's ambition of 100% universal health coverage can be attainable through firm policies and continued government investment in Ayushman Bharat in the upcoming years.⁴²



Lack of human resources has led to underutilization of health research budget. Thus, moving forward, budget allocation (in 2023-24) was increased by 9% for human resource and capacity building. Better utilization of funds can lead to health research enhancement.⁴



ABDM's aim is shifting from "illness" to "wellness", "facility centric" to "patient-centric" and "platform-centric" to "network-centric". These efforts aim to enhance citizens' access to healthcare through advancements like telemedicine and ensure continuity of care by granting individuals access to their digital health records.⁶⁰



As the share of OOPE is notably higher as compared to government expenditure for secondary and tertiary care, standing committee on health has recommended reservation of 25% of cancer facilities for PMJAY in private hospitals. Government run oncology treatment could be expanded as well.⁴



Accredited Social Health Workers (ASHA) programme's future ahead³⁰

- Revising Community Processes Guideline
- Revising strategy-wise support to states for rolling out ASHA certifications
- Encouraging states to utilize community participation platforms



Medical value travel by foreigners in India has increased to 3.04 Lakh in 2021 from 1.83 Lakh in 2020 making India to rank 10th in Medical Tourism index. Many of India's facilities have accreditation and offer care that meets or exceeds international standards and likely to further grow.⁶¹



MoHFW recently launched National Sickle Cell Anaemia Elimination Mission with the aim to screen 7 crore people suffering with SCD especially in tribal areas. More initiatives are likely to follow for addressing issues affecting unserved population.⁶²



During 2021-22, an export of ₹1, 74,995 Crore was recorded in Indian Pharmaceuticals.²⁷ Sales growth is attributed to rise in the capacity of vaccine manufacturing and API manufacturing, which will help further grow the export market.



Government's strategic procurement from private sector to tackle the critical gaps in the public health facilities, would boost the private sector and advance public health goals. National Health Policy 2017, encourages working together with private providers, especially those in under-served areas, to serve the community better. The approach would enhance disease reporting and monitoring activities, sharing and support developing a seamless health information system.²

⁶⁰ NITI Aayog. (2018). *National Health Stack: Strategy and Approach*.
⁶¹ Invest India. (n.d). *Medical Value Travel*. Retrieved 1 September 2023.

Best Practices

1. “TAARE ZAMEEN PAR”: A Night Vigil, Chandigarh⁶²

Background:

- Chandigarh wanted to increase immunisation services at the doorsteps for Chandigarh's nomads and increase the overall immunization rate.
- The Health Department has noted that the health functionaries are unable to provide for the needs of nomads, rag pickers, and beggars who are not stationary and cannot be covered by the micro plans.

Steps taken by the Government

- Teams were dispatched to the locations where nomads and their children spent the night, and vaccinations were administered to any eligible children.
- Team of male police officers, male medical officers, and male health professionals who work in collaboration with the police department equipped with torches, vaccination carriers, and markers were sent to carry out the action.

Impact generated

- Overall coverage improved by almost reach out to more than 300 left outs/ uncovered during each round of Pulse Polio Immunization round.
- The initiative was able to reach the "Unreached Population" and beneficiaries who are not covered and could act as carriers.



Fig 17 – Immunisation services at doorsteps

⁶² Ministry of Health and Family Welfare. (n.d). *National Sickle Cell Anaemia Elimination Mission About Mission*. Retrieved 1 September 2023.

2. Joining hands with Community for Quality Health Services at Health and Wellness Centers, Nagaland⁶³

Background:

- Nagaland had very low rate of institutional delivery, which is 33% as per NFHS-4 data. Due to a lack of a functioning labour room, a lack of qualified HR, community belief in home delivery, and inadequate transportation facilities in the difficult terrain, the Sub Centre's current infrastructure is not suitable for conducting deliveries at the facilities.

Steps taken by the Government

- Village chiefs and other powerful people were educated on the need of community involvement in Health and Wellness Centres (HWCs) as a means of enhancing health results.
- It was the responsibility of the Village Health Committees (VHCs) to convert the existing sub centres into HWCs. These committees make ensuring that current Sub-Centres are built or upgraded and that funding transparency is maintained.
- The building of a communal kitchen for use by family members next to HWC was also funded in part by VHCs.

Impact generated

- Since many of the local priests and other influential members were part of health committees, the majority of the upgrading work has been done in full transparency. Village councils also act as a body for monitoring any development in the village.
- 82,727 residents in the catchment areas of these 49 HWC-SHCs facilities have benefited from this work.
- 49 HWC SHCs feature an adjacent community kitchen that was built with community involvement, and 19 of the 49 operational HWC-SHCs now offer institutional delivery.



Fig 18 - Community involvement

⁶³ National Health Systems Resource. (2019). *We Care - Good, Replicable & Innovative Practices*.

3. Shramyogi Swasthya Sewa, Daman and Diu and Dadra and Nagar Haveli⁶³

Background:

Daman and Diu and Dadra and Nagar Haveli have had significant industrial expansion over the past 20 years. Due to low levels of awareness of medical services that are available, language obstacles, low socioeconomic status, etc. immigrants, who make up a sizeable section of the workforce, are more likely to conceal illness and occupational disorders.

Steps taken by the Government

- "Shramyogi Swasthya Sewa" offers industrial labourers healthcare right at their front doors. This consists of a mobile medical unit in the UT that was provided as part of CSR by the Daman and Diu and Dadra and Nagar Haveli industrial units.
- The Health Department set up the necessary staff, which included a doctor, paramedic, and chemist.
- Vehicles were properly branded to raise awareness of the facility among the intended audience.

Impact generated

Daily 100-150 workers are availing these services. 36,229 beneficiaries in UT of Daman and Diu have availed services till now. More analysis of industrial laborers would lead to identify disease burden of occupational and other diseases for policy corrections to spread awareness among such the target group.



Fig 19 - Services given to workers

Recommended Readings

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